

HP TRAMADOL SR TABLET 100 mg

VITRafa-PC (MY)

DESCRIPTION

White, round, transparent film-coated tablet with shallow convex faces and scored on one face.

COMPOSITION

Each tablet contains: Tramadol Hydrochloride 100 mg

PHARMACODYNAMICS

Tramadol is a centrally-acting analgesic that is not chemically related to opiates. The mechanism of action of tramadol has not been fully determined, but it may bind to mu-opioid receptors and inhibit the reuptake of norepinephrine (NE) and serotonin. The ability of tramadol to inhibit the neuronal uptake of monoamines in the same concentration range at which it binds to mu-opioid receptors differentiates it from typical opioids.

PHARMACOKINETICS

Absorption: Tramadol is readily absorbed after oral doses but is subject to first-pass metabolism. Mean absolute bioavailability of 100-mg dose is approximately 75%. The rate or extent of absorption is not significantly affected by administration with food.

Distribution: Tramadol is widely distributed, crosses the placenta, and appears in small amounts in breast milk.

Protein Binding: Low (20%)

Biotransformation: Tramadol is metabolised by N- and O-demethylation via the cytochrome P450 isoenzymes CYP3A4 and CYP2D6 and glucuronidation or sulfation in the liver. The metabolite O-des-methyltramadol is pharmacologically active. The inactive metabolites are formed by N-demethylation.

Elimination: Tramadol is excreted mainly in the urine, predominantly as metabolites. The elimination half life after oral doses is about 6 hours.

INDICATIONS

HP Tramadol SR Tablet 100 mg is indicated for the treatment of moderate to severe pain.

CONTRAINDICATIONS

It is generally contraindicated in the following conditions:

- Respiratory depression, especially in the presence of cyanosis and excessive bronchial secretion.
- In the presence of acute alcoholism, head injuries and conditions in which intracranial pressure is raised.
- It should not be used in patients receiving Monoamine Oxidase (MAO) inhibitors.
- Previous hypersensitivity to HP Tramadol SR Tablet 100 mg.
- It should not be used during pregnancy.
- It should not be administered during lactation as tramadol and its metabolites have been detected in breast milk.
- Children younger than 18 years to treat pain after surgery to remove the tonsils and/or adenoids.
- Adolescents between 12 and 18 years who are obese or have conditions such as obstructive sleep apnea or severe lung disease, which may increase the risk of serious breathing problems.

PRECAUTIONS

It should be used with extreme caution in patients with the following conditions:

- Decreased respiratory reserves; and should not be given during an attack of bronchial asthma or in heart failure secondary to chronic lung disease, hypothyroidism, adrenocortical insufficiency, impaired kidney or liver function, prostatic hypertrophy, shock or inflammatory or obstructive bowel disorders, myasthenia gravis.
- It should be given with great care to infants, especially neonates.
- The administration of opioid analgesics during labour may cause respiratory depression in the newborn infant.
- Even when administered according to instructions, the preparations may affect the reaction ability of the patients to such an extent that his capacity to drive or operate machines may be impaired. This applies particularly in conjunction with alcohol.
- **Paediatric population:** The safety and efficacy of HP Tramadol SR Tablet 100 mg has not been studied in the paediatric population. Therefore use of HP Tramadol SR Tablet 100 mg is not recommended in patients under 12 years of age.
- **Respiratory depression:** Administer HP Tramadol SR Tablet 100 mg cautiously in patients at risk for respiratory depression, including patients with substantially decreased respiratory reserve, hypoxia, hypercapnia, or pre-existing respiratory depression, as in these patients, even therapeutic doses of HP Tramadol SR Tablet 100 mg may decrease respiratory drive to the point of apnea. In these patients, alternative non-opioid analgesics should be considered. When large doses of tramadol are administered with anaesthetic medications or alcohol, respiratory depression may result. Respiratory depression should be treated as an overdose. If naloxone is to be administered, use cautiously because it may precipitate seizures.

- **Cytochromes P450 (CYP) 2D6 Ultra-Rapid Metabolism:** Some individuals may be CYP2D6 ultra-rapid metabolisers. These individuals convert tramadol more rapidly than other people into its more potent opioid metabolites O-desmethyltramadol (M1). This rapid conversion could result in higher than expected opioid-like side effects including life-threatening respiratory depression. The prevalence of this CYP2D6 phenotype varies widely and has been estimated at 0.5 to 1% in Chinese, Japanese and Hispanics, 1 to 10% in Caucasians, 3% in African Americans and 16-28% in North Africans, and Arabs. Data are not available for other ethnic groups.
- **Risk from Concomitant Use with Benzodiazepines:** Profound sedation, respiratory depression, coma, and death may result from concomitant use of HP Tramadol SR Tablet 100 mg with benzodiazepines. Observational studies have demonstrated that concomitant use of opioids and benzodiazepines increases the risk of drug-related mortality compare to use of opioids alone. Because of these risks, reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate.

If the decision is made to newly prescribe a benzodiazepine and an opioid together, prescribe the lowest effective dosages and minimum durations of concomitant use.

If the decision is made to prescribe a benzodiazepine in a patient already receiving an opioid, prescribe a lower initial dose of the benzodiazepine than indicated in the absence of an opioid, and titrate based on clinical response.

If the decision is made to prescribe an opioid in a patient already taking a benzodiazepine, prescribe a lower initial dose of the opioid, and titrate based on clinical response.

Follow patients closely for signs and symptoms of respiratory depression and sedation. Advise both patients and caregivers about the risks of respiratory depression and sedation when HP Tramadol SR Tablet 100 mg is used with benzodiazepines. Advise patients not to drive or operate heavy machinery until the effects of concomitant use of the benzodiazepine have been determined. Screen patients for risk of substance use disorders, including opioid abuse and misuse, and warn them of the risk for overdose and death associated with the use of benzodiazepines (See Interactions with Other Medicaments).

- **Serotonin Syndrome with Concomitant Use of Serotonergic Drugs:** Cases of serotonin syndrome, a potentially life-threatening condition, have been reported during concurrent use of HP Tramadol SR Tablet 100 mg with serotonergic drugs (See Interactions with Other Medicaments). This may occur within the recommended dosage range.

Serotonin syndrome symptoms may include mental-status changes (e.g. agitation, hallucinations, coma), autonomic instability (e.g. tachycardia, labile blood pressure, hyperthermia), neuromuscular aberrations (e.g. hyperreflexia, incoordination) and/or gastrointestinal symptoms (e.g. nausea, vomiting, diarrhoea) and can be fatal (See Interactions with Other Medicaments). The onset of symptoms generally occurs within several hours to a few days of concomitant use, but may occur later than that. Discontinue HP Tramadol SR Tablet 100 mg if serotonin syndrome is suspected.

- **Adrenal Insufficiency:** Cases of adrenal insufficiency have been reported with opioid use, more often following greater than one month of use. Presentation of adrenal insufficiency may include non-specific symptoms and signs including nausea, vomiting, decreased appetite, fatigue, weakness, dizziness, and low blood pressure. If adrenal insufficiency is suspected, confirm the diagnosis with diagnostic testing as soon as possible. If adrenal insufficiency is diagnosed, treat with physiologic replacement dosing of corticosteroids. Wean the patient off of the opioid to allow adrenal function to recover and continue corticosteroid treatment until adrenal function recovers. Other opioids may be tried as some cases reported use of a different opioid without recurrence of adrenal insufficiency. The information available does not identify any particular opioids as being more likely to be associated with adrenal insufficiency.
- **Sexual Function/ Reproduction:** Long term use of opioids may be associated with decreased sex hormone levels and symptoms such as low libido, erectile dysfunction, or infertility (See Postmarketing Experience)
- **Information for the patients:** HP Tramadol SR Tablet 100 mg is a potent drug for the relief of pain, e.g. in wound pain, fractures, severe nerve pain, tumour pain, heart attack. It should not be used for minor pain. The effect sets in quickly and lasts for some hours.

PREGNANCY AND LACTATION

Pregnancy: Tramadol has been shown to cross the placenta. There are no adequate and well-controlled studies in pregnant women. Safe use in pregnancy has not been established. HP Tramadol SR Tablet 100 mg is not recommended for pregnant women.

Lactation: Approximately 0.1% of the maternal dose of tramadol is excreted in breast milk. In the immediate post-partum period, for maternal oral daily dosage up to 400 mg, this corresponds to a mean amount of tramadol ingested by breast-fed infants of 3% of the maternal weight-adjusted dosage. For this reason, tramadol should not be used during treatment with tramadol. Discontinuation of breast-feeding is general not necessary following single dose of tramadol.

DRUG INTERACTION

It should be given with extreme caution in patients taking monoamine oxidase inhibitors or within 14 days of stopping such treatment.

- It should not be combined with a MAO inhibitor, or within 14 days of discontinuation of it.
- The depressant effects of HP Tramadol SR Tablet 100 mg are enhanced by depressants of the central nervous system such as alcohol, anaesthetics, hypnotics and sedatives, tricyclic anti depressants and phenothiazines.
- Opioid analgesics with some antagonist activity such as buprenorphine, butorphanol, nalbuphine or pentazocine may precipitate withdrawal symptoms in patients who have recently used pure agonists such as tramadol.
- On the concomitant administration of HP Tramadol SR Tablet 100 mg with substances which also act on the central nervous system (e.g tranquilizers, hypnotics) the sedative effects (fatigue) may be intensified. At the same time, however, combining HP Tramadol SR Tablet 100 mg with a tranquilizer, for example, will probably have a favourable effect on the pain sensation.
- Tramadol should not be used in patients receiving MAO inhibitors.
- **Serotonergic Drugs:** The concomitant use of opioids with other drugs that affect the serotonergic neurotransmitter system has resulted in serotonin syndrome. If concomitant use is warranted, carefully observe the patient, particularly during treatment initiation and dose adjustment. Discontinue HP Tramadol SR Tablet 100 mg if serotonin syndrome is suspected. Examples of serotonergic drugs are selective serotonin reuptake inhibitors (SSRIs), serotonin and norepinephrine reuptake inhibitors (SNRIs), tricyclic antidepressants (TCAs), triptans, 5-HT₃ receptor antagonists, drugs that affect the serotonin neurotransmitter system (e.g mirtazapine, trazodone, tramadol), monoamine oxidase (MAO) inhibitors (those intended to treat psychiatric disorders and also others, such as linezolid and intravenous methylene blue) (See Precautions)
- **Benzodiazepines:** Due to add additive pharmacologic effect, the concomitant use of opioids with benzodiazepines increases the risk of respiratory depression, profound sedation, coma and death.

The concomitant use of opioids and benzodiazepines increases the risk of respiratory depression because of actions at different receptor sites in the central nervous system that control respiration. Opioids interact primarily at μ -receptors, and benzodiazepines interact at GABAA sites. When opioids and benzodiazepines are combined, the potential for benzodiazepines to significantly worsen opioid-related respiratory depression exists.

Reserve concomitant prescribing of these drugs for use in patient for whom alternative treatment options are inadequate (see Precautions).

Limit dosage and duration of concomitant use of benzodiazepines and opioids, and follow patients closely for respiratory depression and sedation.

MAIN SIDE/ADVERSE EFFECTS

- Tramadol can produce drug dependence of the mu-opioid type and may potentially be abused. Tolerance development, drug-seeking behavior and craving have been associated with the use of tramadol.
- The commonest side effects include abdominal or stomach pain, anorexia, asthenia, central nervous system (CNS) stimulation, constipation, diarrhea, dizziness or vertigo, drowsiness, dry mouth, dyspepsia, headache, nausea, pruritus, skin rash, sweating and vomiting.
- Rarely respiratory depression occurs.
- **Postmarketing Experience:**
 - Serotonin Syndrome (See Precautions)
 - Adrenal insufficiency (See Precautions)
 - **Androgen deficiency:** Cases of androgen deficiency have occurred with chronic use of opioids Chronic use of opioids may influence the hypothalamic-pituitary-gonadal axis, leading to androgen deficiency that may manifest as low libido, impotence, erectile dysfunction, amenorrhea, or infertility. The causal role of opioids in the clinical syndrome of hypogonadism is unknown because the various medical, physical, lifestyle, and psychological stressors that may influence gonadal hormone levels have not been adequately controlled for in studies conducted to date. Patients presenting with symptoms of androgen deficiency should undergo laboratory evaluation.

- **Infertility:** Chronic use of opioids may cause reduced fertility in females and males of reproductive potential. It is not known whether these effects on fertility are reversible.

OVERDOSE

Clinical features: Cold, clammy skin; confusion; convulsions; severe dizziness; severe drowsiness; pinpoint pupils of eyes; slow heartbeat; slow or troubled breathing; unconsciousness; severe weakness.

Treatment: Intensive supportive therapy may be required to correct failure and shock. In addition, the specific antagonist naloxone hydrochloride is used to counteract very rapidly the severe respiratory depression and coma produced by excessive doses of opioid analgesics. Naloxone may also be given by subcutaneous or intramuscular injection or by intravenous infusion. Administer naloxone with caution because it may precipitate seizures.

DOSAGE AND ADMINISTRATION

The dosage should be adjusted to the intensity of the pain and the sensitivity of the individual patient. Unless otherwise prescribed, HP Tramadol SR Tablet 100 mg should be taken as follows - with or without meals:

Single dose for adults and adolescents over 12 years of age:

1-2 tablets (100-200 mg tramadol hydrochloride) should be swallowed whole (not divided or chewed) with sufficient liquid, preferably mornings and evenings. If in severe pain the analgesic demand is likely to be higher, 2 tablets (200mg tramadol hydrochloride) can be taken as initial dose.

The lowest analgesically effective dose should generally be selected. Daily doses of 400mg tramadol hydrochloride should not be exceeded, except in special clinical circumstances. The dosage interval should not be less than 8 hours.

Children: On account of the dosage strength, HP Tramadol SR Tablet 100 mg are not recommended for children below the age of 12 years.

Paediatric population: The safety and efficacy of HP Tramadol SR Tablet 100 mg has not been studied in the paediatric population. Therefore use of HP Tramadol SR Tablet 100 mg is not recommended in patients under 12 years of age.

Geriatric patients: A dose adjustment is usually not necessary in elderly patients (up to 75 years) with no clinically manifest hepatic or renal insufficiency. In old patients (above the age of 75 years) elimination may be prolonged. Therefore, if necessary the dosage intervals are to be extended according to the patient's requirements.

Hepatic and renal insufficiency/dialysis: In patients with severe renal and/or hepatic insufficiency HP Tramadol SR Tablet 100 mg should not be administered. In less severe cases prolongation of the dosage interval should be considered.

Duration of Treatment: HP Tramadol SR Tablet 100 mg must not be given for longer than therapeutically absolutely necessary. If long-term pain treatment is necessary, checks should be carried out at regular and brief intervals (if necessary with breaks in treatment) as to whether and in what doses further treatment with HP Tramadol SR Tablet 100 mg is necessary.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage:
Store below 30°C.
Protect from light and moisture.

Presentation/Packing:
Blister pack of 3 x 10's.

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